

## **Final Proposal**

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SOWK 352-003: Social Work & Sci Inquiry

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27 April 2025

## **Part I: Problem Statement and Practice-Based Question**

### **Purpose**

Access to safe and legal abortion is increasingly restricted in many regions of the United States due to legislative barriers and misinformation. These restrictions disproportionately impact marginalized groups, including low-income individuals, people of color, and those in rural areas. The lack of abortion access and other reproductive healthcare options leads to significant negative physical, mental, and socioeconomic consequences. Research has shown that individuals denied abortions face higher risks of adverse health outcomes, increased maternal mortality, and higher rates of poverty. Additionally, these restrictions burden healthcare providers, leading to diminished care and limiting reproductive autonomy by preventing individuals from making decisions about their own bodies. One significant consequence of abortion restrictions is the rise in self-managed abortions, which often result in unsafe procedures, long-term health issues, or even death. The financial burden of carrying an unwanted pregnancy to term can also lead to job loss and financial hardship. Beyond individual costs, the increase in pregnancy complications contributes to a higher rate of hospitalizations and emergency care, straining healthcare resources that could otherwise be allocated to saving lives. Given these concerns, this research aims to explore the real effects of abortion restrictions on reproductive choices and health outcomes.

### **Significance of the Proposed Research**

Abortion restrictions and bans are significant both socially and politically. These bans raise concerns about women's rights, reproductive—and general—autonomy, health and safety, social and economic impacts, and political and legal influences. Ensuring access to safe reproductive healthcare is crucial in preventing health complications and providing necessary

resources such as travel assistance, childcare, and financial support. Abortion bans often reflect broader political ideologies, with certain states enforcing stricter laws that are connected to gender inequality and the stripping of reproductive rights. Additionally, religious influences can play a role in shaping policies, often leading to stricter abortion bans in regions with higher maternal mortality and morbidity rates. Further research is necessary because abortion restrictions directly impact women's health, safety, and freedoms. The issue of reproductive rights is highly relevant to social work, as it aligns with the core values of social justice and service, as well as autonomy. Advocating for fair and equal access to reproductive healthcare is essential in ensuring everyone receives the care they need. Agencies can improve their practice by staying informed on restrictions, providing trauma-informed care, offering counseling services, advocating for policy changes, adjusting resource allocation, and training staff to be knowledgeable on current laws and restrictions.

### **Research Question**

Our research question is "How do state-level restrictions on abortion impact individuals' reproductive choices and health outcomes?" This question is clear and specific, allowing us to explore the real effects of abortion restrictions on access to care. Given the limited timeframe for conducting our study, we must be realistic about our constraints and plan accordingly. As college students with varying schedules, we will focus on a smaller-scale study using existing data, online surveys, or structured interviews rather than long-term research. Cost is another major factor that we need to take into consideration. As we lack funding for extensive data collection, we will use free online tools for survey distribution and data analysis. While outside assistance would be beneficial, we will do our best to work within our available resources, potentially utilizing on-campus facilities and programs for research support. Logistically, we may require

help with survey distribution and data analysis, but we will keep the study manageable by using established research frameworks and secondary sources. Partnering with organizations focused on reproductive health could provide valuable insights without adding excessive logistical challenges. Ethical considerations are also key, as this is a sensitive topic. We will implement strict ethical guidelines in data collection and analysis to protect participant confidentiality and ensure informed consent.

### **Hypothesis**

Total abortion bans lead to increased rates of unsafe abortions, pregnancy-related complications, and higher infant and maternal mortality rates. History has shown that even without access to safe abortion practices, individuals facing difficult circumstances will seek alternatives that may result in serious injury or death. Current medical trends indicate that states with stricter abortion laws have rising cases of sepsis due to delays or refusals in treating miscarriages. Texas, for example, has reported increased cases of septic infections due to fears among healthcare providers of legal repercussions. Many doctors, fearing legal consequences and the potential loss of licensure, hesitate to provide essential medical care, even in cases where intervention is necessary for the patient's well-being. Numerous reports highlight healthcare professionals voicing concerns about the risks of medical decisions being misinterpreted as deliberate terminations. By analyzing these factors, this research will provide valuable insights into the tangible effects of abortion restrictions on public health and reproductive autonomy, contributing to the broader discourse on reproductive rights in contemporary society.

## **Part II: Literature Review**

### **Introduction**

Access to abortion is an important aspect of reproductive healthcare, yet it has become increasingly restricted in the United States, due to state-level legislation that undermines bodily autonomy and endangers public health. Abortion restrictions—including mandatory waiting periods, parental consent laws, gestational bans, and targeted regulation of abortion providers—have intensified since the early 2010s, culminating in significant shifts in access, even before the Supreme Court’s 2022 decision in *Dobbs v. Jackson Women’s Health Organization*—which overturned the ruling in *Roe v. Wade* (1973). These restrictions disproportionately affect individuals in marginalized communities, particularly low-income people, people of color, and those living in rural areas (Prather et al., 2018)

Research has shown that state-level abortion restrictions have contributed to unequal access to reproductive healthcare and worsened health outcomes, especially in states with historically aggressive anti-abortion legislation. For example, Bossick et al. (2020) found that states enacting multiple forms of restrictions experienced decreased utilization of reproductive health services, including contraception and preventive care. These patterns suggest a broader public health impact of abortion legislation that extends beyond the procedure itself.

This paper explores how abortion restrictions in the United States affect reproductive health outcomes, with a focus on maternal health, healthcare access, and regional disparities. Specifically, it aims to understand how policy environments shape health risks and limit reproductive choices for individuals in highly restrictive states. The sources were found by searching the University of South Carolina library, EBSCOhost, JSTOR, and other academic

websites, using terms related to reproductive health and policy, as well as looking at specific legislative cases, such as *Roe v. Wade* and *Dobbs v. Jackson Women's Health Organization*.

## **Literature Review**

### **The Public Health Impact of Abortion Restrictions**

Bossick et al. (2020) conducted a review that highlights the broader public health consequences of state-level abortion restrictions. This study examined how laws that target abortion access can also lead to decreased utilization of other reproductive healthcare services, including contraception, cancer screenings, and prenatal care. It included multiple peer-reviewed articles and identified multiple mechanisms by which these restrictions contribute to adverse health outcomes beyond the procedure itself.

One key finding from the review was that restrictive environments often correlate with reduced access to general reproductive health services. For instance, Bossick et al. (2020) describe how targeted regulation of abortion providers can result in clinic closures, which not only affect abortion access but also limit the availability of preventative services, which are often also offered by these same clinics. These closures and limitations disproportionately affect individuals in rural or underserved communities, who may entirely lose access to care if there are no other local alternatives.

The authors also emphasized how the cumulative effect of multiple restrictions in a single state can compound access issues broadly. In states where laws require multiple clinic visits or mandatory waiting periods, logistical barriers such as transportation costs, time off work, and childcare needs become significant deterrents to care (Bossick et al., 2020). These compounded barriers often result in delayed or forgotten medical services, leading to worse reproductive health outcomes and exacerbating health disparities already present in the United States'

healthcare system. Overall, the research suggests that abortion restrictions should not be viewed in isolation, but as part of a broader policy context that shapes reproductive health outcomes. These findings underscore the need to consider the full spectrum of services affected when evaluating the impact of restrictive legislation.

### **US Abortion Bans and Infant Mortality**

The article *US Abortion Bans and Infant Mortality* by Gemmill, Franks, Anjur-Dietrich, and colleagues offers compelling evidence that post-*Dobbs* abortion restrictions are contributing to increased infant mortality in the U.S., especially among vulnerable populations. Building on earlier studies linking reduced abortion access to poor infant outcomes (Myers, 2017; Stevenson, 2019), this study uses a Bayesian panel model to examine state-level changes in infant deaths following abortion bans. The researchers find that states with strict bans, particularly those without exceptions, experienced measurable increases in infant mortality, mainly from congenital anomalies, which are already the leading cause of infant death.

One of the biggest strengths of this study is its careful approach to data analysis. Unlike earlier studies that just compared before and after the bans, the authors use a Poisson model to look at infant mortality, adjusting for factors like COVID-related disruptions and missing data. They also pool estimates across different groups to deal with the issue of sparse data in perinatal research. This helps make their findings more reliable. In terms of external validity, the study looks at different racial and ethnic groups, and the results show that non-Hispanic Black infants are hit the hardest, which ties into the already existing health disparities.

Still, the study does have some limitations. The short post-ban timeframe makes it hard to capture long-term trends, especially in states outside of Texas, where the impact may take longer to show. Some subgroups also had to rely on imputed data due to suppressed death counts, which

could affect the precision of the findings. To build on this research, future studies should extend the observation period to assess long-term outcomes better and include more representative sampling, especially in states with smaller populations or higher data suppression. It is also helpful to rule out other possible explanations, such as changes in healthcare access or socioeconomic shifts that influence infant mortality. These improvements could help strengthen the evidence base and make the conclusions even more solid.

Overall, this literature painted a clear picture that abortion bans have far-reaching consequences beyond reproductive choice, with immediate implications for infant survival. Advancing the field by offering strong causal evidence and highlighting the unequal burden on marginalized communities.

### **A call for increased abortion education**

The article titled The impact of hostile abortion legislation on the United States maternal mortality crisis: a call for increased abortion education(Kheyfets et al., 2025) goes over the statistics surrounding abortion and its disproportionate impact on minorities, racial and otherwise and those in poverty, but it also looks at how this affects healthcare and healthcare professionals. This article highlights the lack of abortion education in medical schools, stating "17% of medical schools in the US did not formally teach abortion, and less than 50% of schools dedicated at least one lecture on abortion" (Kheyfets et al., 2025). It additionally discusses the decrease in people choosing to go into obstetrics and gynecology, especially in states with strict abortion laws.

What this all means is that there is a decrease in access to a medical procedure that can be lifesaving, not only due to laws directly restricting access. It also causes issues accessing ob/gyn services because a lack of abortion providers means a lack of ob/gyn providers, because abortion is sometimes a necessary part of obstetric care. Due to the lack of providers, people in



underserved areas are often forgoing prenatal care, routine medical screenings, needed gynecological care, and more. Lack of prenatal care increases both infant and maternal mortality rates. Because the populations that are underserved are frequently minorities or people living in poverty, the already high maternal mortality rate for these populations is only increasing. The mortality rate for black birthing people could increase as much as thirty-three percent under a total abortion ban (Kheifets et al., 2025).

The impact of hostile abortion legislation on the United States' maternal mortality crisis: a call for increased abortion education is not solely focused on straight statistics, though. It also proposes ideas for solving this increasing disparity. It puts forth the idea of medical schools and their accreditation boards cementing abortion care into their curriculum. It highlights the need to diversify the medical field so people can have doctors who understand their lived experience and so doctors can work with people from diverse backgrounds, helping to increase their cultural humility, competence, and respect. Finally, it highlights the need to codify into law the right to access abortion care.

### **Abortion Restrictiveness and Infant Mortality: An Ecologic Study, 2014-2018**

Examining how strict reproductive policies affect diverse countries like the United States is important in highlighting the significance of implementing more protections for reproductive rights. Factors such as race, class, and access to resources influence the well-being of infants affected by these laws. Also, disparities in prenatal and birthing care exist among different demographic groups as a result of such policies.

A key example of this correlation is examined in the article "Abortion Restrictiveness and Infant Mortality: An Ecologic Study, 2014–2018" by Burdick et al. (2023). The study investigates the association between state-level abortion restrictions and infant mortality rates

(IMR) in the U.S. from 2014 to 2018. Using publicly available data from the CDC WONDER database, a national public health resource under the Department of Health and Human Services, the researchers conducted an ecologic, or population-level, analysis. This data is valuable as it minimizes bias, unlike privately funded studies that may be influenced by outside interests.

States were categorized based on abortion restrictiveness using the Guttmacher Institute's classification: restrictive, moderately restrictive, and protective; protective refers to states with the least restrictive reproductive health policies, particularly concerning abortion. The study found that states with more restrictive abortion laws had significantly higher infant mortality rates compared to those with protective policies. Even after adjusting for variables that influence both the dependent and independent variables, there was a 12% higher infant mortality rate in states with restrictive laws. Especially, non-Hispanic Black infants experienced disproportionately higher mortality rates, highlighting a racial disparity; particularly in southern states, which tend to have stricter reproductive laws and larger Black populations. These findings suggest that restrictive abortion policies may worsen infant health outcomes, especially in marginalized communities.

However, the study has weaknesses. Relying on state-level CDC and Guttmacher data may not take into account sub-state variations and overlook individual-level factors such as maternal health, prenatal care access, or barriers to abortion. Also, the Guttmacher classification does not account for exceptions or enforcement variability in abortion laws, potentially leading to misclassification. The study also excludes maternal mortality and pregnancy-related morbidity, which are important to understanding the full impact of restrictive reproductive laws. In addition, it does not address disparities among other marginalized racial groups or low-income

Despite these limitations, the study demonstrates that restrictive reproductive laws were already a significant issue, particularly for marginalized groups, even before the overturning of *Roe v. Wade* (1973). The findings reveal a persistent correlation between abortion restrictions and higher infant mortality rates, even after adjusting for variables that influence both the dependent variable and independent variable. By analyzing a large, diverse population using credible federal data, the study strengthens the validity of its conclusion.

### **Addressing the Gaps of Reproductive Health Policy Impacts**

The article *Impact of State-Level Reproductive Health Legislation on Access to and Use of Reproductive Health Services and Reproductive Health Outcomes: A Systematic Scoping Review in the Affordable Care Act Era* explains the relationship between health care outcomes and state-level reproductive health policies. Additionally, this article goes into depth to describe the impacts that policies have on reproductive health outcomes for women over the age of eighteen. The research from this article focuses on two state-level policies that interact with and influence outcomes under federal policy, which are the Patient Protection and Affordable Care Act.

Several limitations were found in the literature that point to deficiencies that the research should address in the future. This study identified eight studies that met the criteria, with only half focusing on restrictive abortion legislation. Only two studies covered all 50 states, while the other studies focused on single states. This focus narrows the generalizability of findings and underlines more comprehensive studies. This review excluded qualitative studies on how reproductive health policies affect individuals' experiences. The studies examined had isolated policies that didn't aim to evaluate the impact of various policies that better reflect real-world

scenarios. These gaps are important to note, so that state-level reproductive health legislation can advance and shape access to care and health outcomes.

This study builds on the structural policies that intersect with social determinants of health, education level, and reproductive health outcomes. This can support the development of more inclusive policies and outreach strategies that help those in need. By identifying the limitations of the literature, this study offers a more inclusive and comprehensive approach. By using state-level policy analysis and individual-level survey and interview data, the study will be impacted by macro-level policies and micro-level experiences. This article lays the foundational framework that restrictive reproductive health policies, including abortion regulations are associated with negative health outcomes.

### **Abortion in the US: Barriers to Access**

Even though abortion has been legal in the U.S. for decades, actually getting an abortion is still extremely difficult for many people, especially for people who are low-income, young, people of color, and people living in rural areas. In her article *Abortion in the US: Barriers to Access*, Marlene Gerber Fried explains how political decisions, lack of funding, and stigma have made it harder and harder for people to access safe and legal abortion care. She points out that even though the procedure itself is safe and relatively low-cost, it is still often not accessible for individuals who cannot afford it or who live far from a provider.

Fried talks about how laws like the Hyde Amendment cut off funding for abortions through Medicaid, which makes it nearly impossible for many low-income people to afford care. It forces many women to delay care or carry unwanted pregnancies to term because they cannot afford the procedure. This has led to more people being forced into second-trimester procedures, trying to administer abortions on their own in unsafe ways, and facing higher risks to both

maternal and infant health. There is also a huge shortage of doctors willing or able to offer abortions, partly because of threats, harassment, and political pressure. This all adds up to a system where reproductive healthcare is technically legal, but completely out of reach for a lot of people.

What is missing from the research is more up-to-date information on how recent total abortion bans and “heartbeat laws” are actually affecting people’s ability to make decisions about their own bodies, get medical care, and deal with the real-life consequences. This is especially prominent in states where Christian nationalism heavily influences policy and laws.

Our research builds on this by looking at how state-level abortion restrictions are actually affecting the choices people with uteruses can make and the risks they face because of it. We want to shine a light on the gaps in access that often lead to serious health complications, higher rates of maternal and infant deaths, and financial struggles. When abortion is totally banned, it does not stop people from needing care, it just leads to more unsafe procedures, higher rates of pregnancy complications, and increased maternal and infant mortality rates.

## **Part III: Methods**

### **Research Design**

A web-based survey is the most appropriate research design for examining the effects of abortion restrictions on reproductive choices and health outcomes. It allows for anonymous participation, which helps ensure safety and comfort when discussing sensitive experiences. Questions could include items such as, “Have abortion restrictions in your state influenced your reproductive health decisions?” along with demographic information like age, location, and income to identify patterns across different populations.

While web-based surveys rely on self-reported data, which may introduce bias, and may underrepresent groups with limited internet access, the benefits outweigh the limitations. This method provides broad geographic reach, is cost-effective, and respects participant privacy—an essential consideration for topics that carry potential stigma or risk. Overall, it is the most feasible and ethically sound approach for this research.

### **Sampling Design**

Regarding the sampling design for this study, our group will aim to sample approximately 250 to 300 participants. This large sample size would ensure a diverse and reasonably representative dataset. While the sampling method is non-probability based, the relatively large sample size will help to strengthen the generalizability of the findings. Additionally, this will allow for comparison across demographic subgroups like age, region, and income level.

Participants will be recruited online, mostly through social media platforms like Facebook, Instagram, Twitter or X, where targeted ads and digital flyers can be used to reach those birthing across the United States. Also, outreach may occur through online support groups,

reproductive health forums, and mailing lists of organizations focused on reproductive justice. This approach ensures broad geographic coverage and offers participants anonymity and convenience. This is essential when discussing sensitive topics such as abortion and reproductive health.

We are using a non-probability sampling method, specifically a combination of purposive and convenience sampling. This approach would allow us to intentionally reach individuals with experiences related to reproductive health, while also making participation accessible through online platforms where individuals can participate voluntarily.

This sampling design is the most appropriate for our study because we are focused on gathering rich, personal insights rather than making broad generalizations. While we recognize that non-probability sampling does have some limitations, such as potential sampling bias and limited representativeness, it aligns with the goals of our research by collecting meaningful, real perspectives. It also offers a practical and ethical way to engage with participants on a sensitive subject, which is accessible through digital recruitment methods.

### **Data Collection**

Data will be collected by analyzing responses to a standardized questionnaire. The dependent variable is the set of questions provided to all participants, while the independent variable consists of their answers, which will reveal how legal restrictions influence decision-making regarding reproductive rights. Data will be collected through a brief, anonymous online questionnaire open to birthing people in all 50 states. The survey will be promoted via social media, including flyers and targeted ads, to ensure broad participation. The intervention is digital, accessible at any time, and designed to take only a few minutes to complete.

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